Adjuvant ribociclib + endocrine therapy

Ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to 3 years. The aromatase inhibitor continues daily on its own schedule; ovarian suppression is added when indicated.

Draft — independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart rhythm / QT

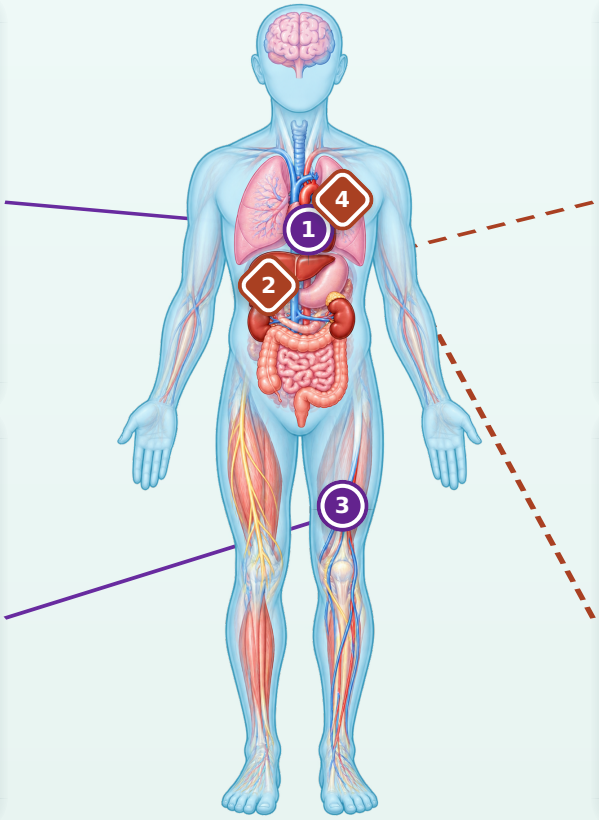
Ribociclib

Baseline ECG and an ECG around Cycle 1 Day 14. Fainting or palpitations need urgent advice.

2 Liver injury

Ribociclib

Keep liver blood tests. Yellow skin / eyes, dark urine or marked upper-abdominal discomfort: call promptly.



3 Low neutrophils / infection

Ribociclib

Blood counts guide whether the next cycle can start. Fever, shivers or sudden illness: contact oncology immediately.

4 Lung inflammation

Ribociclib · uncommon but important

New cough or breathlessness needs prompt assessment, even between visits.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Ribociclib

- Low white cells and infection
- Liver injury / abnormal blood tests
- QT prolongation / heart-rhythm problems
- Nausea, fatigue and hair thinning
- Rare serious lung or skin reactions

Non-steroidal aromatase inhibitor ± ovarian suppression

- Hot flushes, joint pain and fatigue
- Bone loss with AI / ovarian suppression
- Injection-site discomfort with fulvestrant
- Drug-specific liver and other effects

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Support for this phase

Agree symptom medicines with the oncology pharmacist because many anti-sickness and other medicines can affect QT or CYP3A interactions. Do not automatically reuse a chemotherapy antiemetic prescription.

Bloods and cardiac checks

Review prior toxicity → FBC, renal, liver and electrolytes → FBC / renal / liver every 2 weeks. Repeat ECG around Cycle 1 Day 14.

Visits and practical support

Ask about travel, work, fertility, menopause, exercise and help at home. The team arranges and reviews clinical checks; the patient should not have to calculate the monitoring timetable.

Follow-up and bone health

If an aromatase inhibitor or ovarian suppression is prescribed, assess fracture risk and baseline DEXA as indicated. Repeat timing depends on the result, risk and bone treatment; it is not an automatic scan for everyone.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00892/

Sources and review status are available online. Follow your own prescription and hospital instructions.